Nivolumab and ipilimumab for advanced melanoma

Your diagnosis and treatment setting NCCP 00431a: advanced unresectable or metastatic melanoma in adults. This patient guide is the melanoma context, not the separate liver-cancer indication.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Combination induction

Active medicines: Nivolumab + Ipilimumab

Emetogenic risk: low

Medicine	Dose / route	When
Individual low-risk anti-sickness plan	Clinician/pharmacy completion if required	According to the local low-emetic-risk policy No routine immune-toxicity steroid or issued prescription is created by this draft.

Additional phase-specific support

- Combination induction is low emetic risk; nivolumab maintenance is minimal risk. Follow the individual/local anti-sickness plan. Routine premedication is not usually required; steroid immune-toxicity rescue is not routine prophylaxis.
- Discuss fertility and pregnancy precautions before starting. Immunotherapy may harm a pregnancy. Use the contraception plan and after-treatment duration specified for the medicines you receive; ask about breastfeeding restrictions. Tell the team promptly about possible pregnancy. Do not stop necessary hormone replacement or prescribed steroids without advice.
- Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.
- This is not an issued prescription. The clinician and pharmacist must complete and authorise any supportive medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00431 v11 · pSee current source monitoring/supportive-care sections · NCCP regimen and local supportive-care policy

Sources checked 2026-11-06 · SACTCheck v0.84.40 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Nivolumab alone

Active medicines: Nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Combination induction is low emetic risk; nivolumab maintenance is minimal risk. Follow the individual/local anti-sickness plan. Routine premedication is not usually required; steroid immune-toxicity rescue is not routine prophylaxis.
- Discuss fertility and pregnancy precautions before starting. Immunotherapy may harm a pregnancy. Use the contraception plan and after-treatment duration specified for the medicines you receive; ask about breastfeeding restrictions. Tell the team promptly about possible pregnancy. Do not stop necessary hormone replacement or prescribed steroids without advice.
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